“How Advocacy Leads to Better Patient Outcomes and Experiences” (Steven Merlin) [#126]

Brad Power and Emily Kim
January 8, 2025

“You need to do your research, and don't be afraid to speak up if you find things. Present that to your care team. Have the information with you.” – Steven Merlin

“Always write down things. I’m a big advocate of using the patient portal, so that between my follow-up exams, I'll make notes and put it in there to make them aware of how I'm feeling, so that they can then address it when I go there, in case I forget it.” – Steven Merlin

“Stay attuned to your body. If something's not right, don't say, ‘I'm going to wait and see if it goes away.’ If it hasn't cleared up within five days, go see your primary care specialist.” – Steven Merlin

Meeting Summary

One of the core values of the Cancer Patient Lab is advocating for yourself and sharing what you have learned on your cancer journey, so that others who follow can better advocate for themselves, all in the pursuit of better experiences and outcomes.

But what does being an advocate for yourself and others look like?

Consider the story of Steven Merlin. Steven is a 12-year survivor of stage IV (metastatic) pancreatic cancer. He was diagnosed in 2012 and underwent the standard surgery (Whipple) to remove the head of his pancreas, part of his small intestine, gallbladder, bile duct, and lymph nodes. Because metastatic disease was detected a week post-Whipple, the medical center tumor board recommended palliative treatment only. However, having trained as a medical technologist and a career in clinical cancer research, he had a basic knowledge on cancer biology and treatment in general. He embarked on learning as much as he could, using the resources of several professional organizations on the internet, ranging from pancreatic cancer advocacy organizations and websites of the National Cancer Institute (NCI), the American Society of Clinical Oncology (ASCO), and the American Cancer Society. He realized that palliative care was not an aggressive enough treatment for him as his goal was cure, so he advocated for more chemotherapy treatment (FOLFIRINOX) than the standard, to achieve his goals of becoming a long-term survivor, if not cured. He was an exceptional responder to this more aggressive treatment than what was recommended.

From his personal experiences of his treatment and care, Steven realized that there was a need to mentor and educate patients and caregivers on patient advocacy. At first he became active in policy advocacy on behalf of the Pancreatic Cancer Action Network. Serving as a volunteer mentor in the PanCAN.org Survivor and Caregiver program allowed him to see where healthcare was deficient in addressing the basic needs of patients that lead to improving quality of life and better outcomes. To educate patients and caregivers on the importance of self-

advocacy, he is actively engaged on social media sites, PanCAN, and LetsWinPC, drawing attention to the various treatments and procedures that should be explored, and considering clinical trials earlier in one’s treatment that may provide better outcomes.

What lifestyle factors are most helpful in managing pancreatic cancer, and how can you incorporate these into your care?

- **Diet**: Follow a low-fat, non-fat diet, but allow yourself some indulgences like red meat and desserts occasionally – a balanced diet is important.
- **Exercise**: Exercise regularly, aiming for at least 150 minutes per week, through activities like walking, using a stationary bike, and stair climbing.
- **Monitor**: Be attuned to your body. Don't ignore symptoms.
- **Proactive**: Don’t be afraid to advocate for yourself.

What role does genetic testing play in the management of pancreatic cancer, and how often should it be repeated?

- **At diagnosis**: Lean into getting genetic testing, especially if you have a family history of cancer.
- **Treatment selection**: If you find you have a mutation, e.g., BRCA2, it can guide your search for clinical trials.
- **Monitoring**: You should not assume your genetic profile is static. Regular genetic/genomic testing can identify whether your mutations change or the development of new primary cancers, which will impact your treatment options.

How can patients effectively advocate for themselves and stay informed about their treatment options?

- **Be organized**: Keep detailed records, e.g., notebooks with all your lab results, radiology reports, and other medical information. Scan paper records onto your computer to easily access and share your medical history and test results with your care team.
- **Conduct research**: Use online resources like clinical trial databases, medical journals, and AI-powered tools to research treatment options. Proactively look for clinical trials that might be a good fit, especially those targeting your mutations.
- **Communicate**: Don’t be afraid to speak up, question your doctors, and advocate for the treatments you feel are best for you. Be sure to voice your concerns and preferences, even when they differ from the initial recommendations, and make sure that your care team is aware of any changes in your condition. Be persistent.
- **Use patient portals**: Use the patient portal to document your symptoms and progress between appointments.
- **Seek multiple opinions**: Second or third opinions from other cancer specialists can be helpful in confirming treatment plans and exploring additional options.

How can you learn more about advocating for yourself?

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- Contact Steven Merlin at smerlin@pancanvolunteer.org.
- “A Guy with Two Cancers Explores Treatments and Life” (Burt Rosen) [#112]
- "A Marketing Analytics Expert’s Journey Through Testicular Cancer Treatment" (Scott Petinga) [#123]
- “Permission to Be Pissed” (Matthew Zachary) [#69]
- “Self-Advocacy in the Era of Precision Medicine” (Rome Madison) [#133]

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Meeting Notes

KEYWORDS

pancreatic cancer, Whipple procedure, clinical trials, PARP inhibitors, genetic testing, BRCA mutation, self-advocacy, FOLFIRINOX, surveillance scans, patient portal, financial toxicity, clinical research, metastatic disease, patient care, medical technology

SPEAKERS

Steven Merlin (87%), Brad Power (6%), Roger Royse (3%), Rick Davis (2%), Ellen Miller (1%), Bill Paseman (1%), Alane Watkins (0%)

SUMMARY

Steven Merlin, a former medical technologist and clinical researcher, shared his journey with pancreatic cancer. Diagnosed in 2012 with stage four mixed tumor type of acinar cell carcinoma and ductal adenocarcinoma, he underwent a Whipple procedure and 46 cycles of FOLFIRINOX, achieving a 80% tumor shrinkage of the largest tumors and elimination of the smaller tumors He discovered a BRCA2 mutation and enrolled in a clinical trial using the PARP inhibitor rucaparib, which he has taken for over 10 years. Merlin emphasized the importance of self-advocacy, frequent surveillance scans, and staying informed. He also highlighted the role of genetic testing, the benefits of clinical trials, and the need for persistent advocacy in cancer care.

OUTLINE

Steven Merlin's Background and Initial Diagnosis

- Steven Merlin was trained as a medical technologist and transitioned into clinical research at the National Institutes of Health.
- His initial symptoms of pancreatic cancer included darkened urine.
- His primary care physician initially dismissed his concerns.
- His primary care physician referred him to Weill Cornell Medical College, where he was diagnosed with a rare type of pancreatic cancer called acinar cell carcinoma.

Initial Treatment and Advocacy

- Steven underwent a Whipple procedure quickly after his diagnosis, which involved removing the head of the pancreas, part of the bile duct, the gallbladder, and part of the duodenum. He also had portal vein resection due to vascular involvement.
- He was diagnosed with stage four pancreatic cancer.
- His oncologist initially placed him on palliative treatment, but he advocated for himself and switched to a more aggressive treatment called FOLFIRINOX.
- He completed 46 cycles of FOLFIRINOX over 24 months, which led to a significant reduction in the size of his tumors.

Search for Clinical Trials and Genetic Testing

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- Steven began searching for clinical trials to further his treatment, using websites like clinicaltrials.gov and PanCAN.org.
- His persistence paid off when he found a pilot study at the University of Pennsylvania using a PARP inhibitor, which showed promising results in two patients.
- He contacted the National Cancer Institute and the Pancreatic Cancer Action Network to inquire about the trial, eventually securing an interview at the University of Pennsylvania.
- His genetic testing revealed he had a BRCA2 mutation, which led him to focus his trial search on PARP inhibitors.

Advocacy and Self-Advocacy Techniques

- Steven emphasized the importance of being organized and keeping detailed records of his medical history and test results.
- He used various resources, including patient portals and online databases, to stay informed about his condition and potential treatments.
- He advocated for more frequent surveillance scans to monitor for recurrence, which his insurance covered without issue.
- He also involved himself in government advocacy to address issues like the high cost of medications and financial toxicity.

Clinical Trial Participation and Long-Term Survival

- Steven participated in a clinical trial using a PARP inhibitor called rucaparib, which he has been on for over 10 years.
- He experienced some side effects, including anemia, but managed them by reducing his dosage and taking periodic breaks.
- His persistence and advocacy led to his long-term survival, and he continues to monitor his condition with regular scans and blood tests.
- He encourages other patients to be proactive in their care, stay informed, and advocate for themselves.

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TRANSCRIPT

Brad Power
This is the Cancer Patient Lab.

We're honored to have Steven Merlin with us. He's an example of a very engaged patient, and you'll hear his story and how he managed to get actively involved in his care. He's with us here today in a relatively healthy state because of the interventions that he did and the things that he [monitored] at various points in his journey to make certain treatment decisions.

This is for informational purposes only. We like to arm patients and caregivers with information they can take to their medical team. This is not medical advice.

We are a volunteer-led nonprofit and welcome any donations that you could make to our community. You can do that through our website, where there's a Donate button.

Steven is an exceptional patient, and he has an amazing story to tell about his journey with pancreatic cancer.

Steven Merlin 1:39
Thank you, Brad. I'm happy to be here, and thanks to all of you that are attending this meeting today.

I will give you a brief executive summary of my background. I initially trained as a medical technologist from 1979 to 1980 in a local medical center. My first exposure to pancreatic cancer was sitting in the patient Grand Rounds, listening to the case presentations, and thinking to myself, “There's one cancer I never want to have.” Back in 1979, there were generally no drugs for [pancreatic cancer]. If you had a low stage diagnosis and were eligible for the Whipple procedure, you had the best chances of survival, but even then they were still low. After I became a medical technologist, I quickly transitioned into clinical research and began my career at the National Institutes of Health; at one point I was working on a collaborative study with Dr. Anthony Fauci. I started off in immunology and then began to get involved in cancer research.

In 2012, I was working in the Department of Pathology at Weill Cornell Medical College in New York City when I started to get the initial symptoms of pancreatic cancer. After five days of having darkened urine, I decided to go see my primary care physician who pronounced me to be the picture of health. He [checked] my vitals–everything was fine–and he said if I didn't feel better, I should come back at the end of the week. Well, I didn't feel any better, I didn't feel any worse, so I went back and said, “There's something really wrong here.” And having been a med tech, I made some suggestions on doing liver function tests. I was at the age of 55 at the time–my father, at the age of 55, had gallbladder disease. From the presentation of symptoms, I thought, “Maybe this is related to having gallstones.” We did liver function tests and a few other things, and that led to my getting a sonogram.

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When the sonograms showed that there were no gallstones or any sludge in the bile duct, my primary care physician cut the losses, called his colleague at Weill Cornell Medical College where I was working, and asked, “Could [I] send [Steven] over and [you] have a look at [him]?” He told me, “Just head right down to his clinic.”

When I got there, his colleague was waiting for me, and we started to do the testing.I think probably all of us were brought up where you're told, “listen to what the doctor has to say,” and usually we don't question anything. And I was the same way. I got my diagnosis, and I listened to what the proposed treatments were going to be, and everything seemed fine for that. I had a Whipple very quickly. I was diagnosed on Friday at 1:30 PM in the afternoon. I signed all the paperwork, and when I asked the surgeon when he wanted to do the surgery, I thought he would say seven to ten days from that point. Instead, he said, “Tomorrow morning, we'll meet again at 6:30. I said, “Wait a minute, isn't today Friday? You're going to give up a round of golf on a beautiful June Saturday?” He told me that I had a very rare and aggressive form of pancreatic cancer called acinar cell carcinoma. About one percent of people get that.

We put off the surgery until the following Monday. A week after surgery, they did a post-surgery CT scan and found that I didn't have a blockage in my intestine because I wasn't able to have a bowel movement–that's the purpose why they did that CT scan–but they never told me that they found metastatic disease in my liver. The tumor board meets, and the tumor board decides that they're going to put me on palliative treatment, because there was no point in doing curative treatment. I had no idea that I was stage four at this point, and I just assumed that everything was fine, that the doctors were doing what would be in my best interest.

Three months later, the next scan was done, and the results were released before the doctor saw it. This was at a time before they would automatically release your results to your patient portal. On my way to get chemo that morning, when I saw the results and let out a gasp on the train, I got to the clinic, saw my oncologist, and said, “Please don't preface your remarks with ‘I'm sorry.’” He looked at me kind of strangely and said, “What are you talking about?” I showed him my iPhone with the results. He was trying to sugarcoat it [and asked] “Why didn't they put me on the more aggressive FOLFIRINOX (chemotherapy) treatment?” It is four agents that they give you that are very harsh and used in the curative setting. That's when I started to advocate for myself and said, “Look, we did Plan B first instead of Plan A: how about we do Plan A? I've got nothing to lose here, all of my affairs are in order. I understand the repercussions that I could get from doing this extensive chemotherapy at a high dosage, but I'm willing to take the risk. I have nothing to lose.

Thankfully, he agreed. They watched me very carefully, and I tolerated the first dose well. I went through six and was doing well. And then, to give my oncologist credit, he decided, since I was going to be on this for the long term, seeing that I was handling it well, he took away two of the agents: one that causes the peripheral neuropathy in your feet and your hands, and the other one that kind of is very harsh on your GI tract. Then I had six resting cycles of just the backbone agent called 5-fluorouracil and a vitamin called leucovorin (it's a reduced form of folinic acid). I did six of those [and] that gave the peripheral nerves a rest. Then again, for the following six, I

went on full dose. And we kept doing this. They were watching very carefully; they would always ask how I felt, and compared to the alternative of being dead, I'd always say, “I feel great”, even though sometimes I didn't. But I was just so afraid that they would say, “Well, maybe we should take a break here.” Well, I got through it with no breaks. I did 46 cycles every 15 days for 24 months, and I hung in there. And then, during this time, I started to look for clinical trials. I remembered, after coming out of my whipple surgery and how bad pancreatic cancer is, that it was probably going to take a clinical trial to make a difference here. That's when I started looking on my own, and at that point, I was searching blindly. I had no idea what to look for so I would just go on the “clinicaltrials.gov” website, put in a parameter field for “pancreatic cancer” and then “metastatic,”and see what would come up. And you're reading through tens and tens of clinical trials there, trying to find something that might be appropriate.

When I had a CT scan after the first six cycles of FOLFIRINOX and was doing extremely well. I had double the amount of shrinkage that they see in most people. I had 60% shrinkage–they considered 30% after three months really [good]. I was asked, “What was the history of [your] family in regards to cancer?” I had already told the surgeon that months before, so I went over it again that every female on the maternal side of my family had breast cancer. Then I was asked [if I was] ever checked for any kind of genetic mutations. I said, “No, I was only advised, because my mother had colon cancer in addition to breast cancer, that I should start screening for colon cancer earlier than what is normally prescribed,” which I did, but nobody ever mentioned anything about BRCA mutations. I did know that my mother's first cousin and her two daughters had been screened for the BRCA mutation, and they were found to be BRCA2 positive, and I made mention of that to my oncologist, so that triggered me getting tested for the BRCA2 mutation. Being positive for that [test], I immediately called my brother and said “You need to get genetic testing done to see if you have a germline BRCA mutation.” And sure enough, he has one, but he's healthy.

Steven Merlin 10:53
Now that I knew that I had a BRCA2 mutation, that gave me an idea of what to look for in clinical trials. I met with a geneticist to inform me that there were clinical trials that targeted the BRCA mutations and to look for PARP inhibitors. I started to focus my trial search on looking for PARP inhibitors. It took me 14 months. I was very persistent. I would find trials that were close, but not quite a good match. I just hung in there, and I would look about every week. I was on this site of the American Society of Clinical Oncology. They were having their meeting, which they usually do at the end of May, first week of June. I remember going on there on a Saturday to see if there was anything posted on recent advances in pancreatic cancer, and there was nothing. I got up Sunday morning, did the same thing, and there it was: an abstract post that was presented by a group at the University of Pennsylvania, mentioning about a pilot study on two patients where they used a PARP inhibitor. Within six months, there were really dramatic results, and it gave a hint that they were going to open a clinical trial using a PARP inhibitor.

On Monday, I called the National Cancer Institute, because I had looked on “clinicaltrials.gov” for this trial, and I couldn't find it. They looked for [the trial for] me [and] they couldn't find it. But then I called the Pancreatic Cancer Action Network (PanCAN), which is an advocacy group, and

they have an extensive database: they couldn't find it as well. At that point, I told the Pancreatic Cancer Action Network that I had the contact information of the lead author of the abstract that was presented. I had her email and her phone number–[I asked them if they] could contact her and let them know that I was interested in this potential trial.

I got a call back from PanCan about 20 minutes later, and they said they got in touch with people at the University of Pennsylvania. They gave me the name, phone number and email of a clinical trial nurse coordinator. I called her up and told her who I was and the trial I was interested in, and she's drawing a blank. She said, “I have no idea that we're doing any kind of clinical trials for pancreatic cancer. We're doing [trials] on women with ovarian and breast cancer with BRCA mutations.” And I said, “Gee, that's strange. PanCAN called [UPenn], they got connected to a clinical trials department there. That's how I got your name and your phone number. Okay, I'll call PanCAN back.”

This is where [the] persistence and perseverance really comes in when you're doing self advocacy. I asked PanCAN if they would call the [supposed] principal investigator of this trial, which they did. I got a call back from the principal investigator a couple of hours later, who explained to me that they couldn't mention a word of this potential trial to anybody outside of the authors of that abstract until it was published, so she couldn't even tell her clinical trial nurse coordinator, and that's why that person didn't know. I went down, I had the interview, I had all of my records all together. As part of self advocacy, from the very beginning, I was always very organized. And remember, back in 2012, things were just starting to transition to patient portals. What I would do at each of my visits and exams was I would ask for printouts of the exam notes, and I would also get the test results from blood chemistry and anything else I did. Whenever I went for a CT exam, they would give you a complimentary copy of your CT scan on a CD disk. I would always get that. Usually they would give me two copies: I would keep one in my own archive at home here, and I could always burn copies from that on my computer if I needed additional ones. I had notebooks. They were organized into sections where I had all of my lab work, my radiology interpretations, and anything else [so] that I could find things very quickly. I was also scanning them onto the hard drive of my computer so I could easily burn it onto a disc. When I found this clinical trial, I was able to put all [of this] information on the CD and [send it] overnight to the University of Pennsylvania so they had it the next day.

Some of the things I did in self advocacy was a lot of my own research. I would do searches on the internet. We all know that there's a lot of things which are standard of care, but sometimes it might take something beyond standard of care to try to get a better outcome. For each of the different cancers, they'll have a standard protocol of things that they use, but there may be other techniques out there that are currently in use for another cancer, and they might be able to be used for another type of cancer, and that's very true with pancreatic cancers, where techniques that are being used for other solid tumors, such as colorectal cancers [and] liver cancers, can be ported over to pancreatic. I started trying to find out what those things were and read up on them, and now using things like artificial intelligence, ChatGPT, [and] another good one called openevidence.com. [Openevidence.com] is a consortium of a number of leaning medical institutions and some [sponsoring] pharma and biopharma companies. That's really good for

anything that has to do with clinical medicine. And I'll put a query out there. It's important in how you phrase things for the type of information that you're going to get back. The more information you can put in [these AI engines while remaining] clear and concise, the better the information that's generated that you can work with. I was using [AI] to find out about different types of other [potentially viable pancreatic cancer] treatments that are used [and] approved in other cancers but (..) might not yet be FDA-approved for use in pancreatic cancer and that really helped.

I did use standard of care [for the most part], but where we went off a standard of care was [when] I went well beyond the twelve cycles that they normally use for the chemo[therapy] protocol I was on, called FOLFIRINOX. And that clearly made the difference. Another thing that I advocated for is that, over time, as you progress and you have no evidence of disease, they start to cut back on the number of surveillance scans that you do. Well, with pancreatic cancer being so recalcitrant, I opted to do more than just one a year. I had requested that we do [the scans] every six months. [Say,] if something occurred, [like you were] doing just yearly, and a month later you start to get recurrence, you wouldn't know [about it] for a year. And with pancreatic cancer, it could really take hold and it's quite hard to eradicate. Once it comes back, it's usually more aggressive. I felt that if I did [the scans in] an interval of six months, I'd be in a little bit better position to try to then treat it. That was fine. I was told we could do six months. There's been no issues with private insurance. Since then, I've transitioned to Medicare. I've had no problems with Medicare paying for both of those. I frequently go for multiple opinions. I never had any issues when I went for more than one opinion. It's always been paid for, no questions asked.

[In summary], there's a lot of things there. Don't assume that it can't be done. Always ask and find out, and usually the answer is that you're allowed to do that. Those were some of the advocacy things that I did. And then, besides self advocacy, I've been involved with government advocacy, [like] trying to make things better for patients, particularly [the] high cost of medications and financial toxicity that occurs from costs of these things.

Steven Merlin 20:09
I try to make people aware that there's a lot of help out there through organizations and some of these pharmaceutical companies, where the cost may be prohibitively expensive and you may have a very high co-pay. There's patient assisted programs from all the different pharmaceutical companies, which will help you in getting a discount card for a very low co-pay. Or, in some cases, you'll get the product for free, so you won't have to pay for it.

[Another] thing is that, sometimes, people are reluctant about doing clinical trials. One of the barriers is, well, it's too far, or they think it's going to [make a negative] impact. “I've got to work. I need to make a living here, get a salary. I'm getting my health insurance through that.” They either want to take a clinical trial that's very close by, or not do one. What I tell those people is that, number one, you're more valuable to your employer alive than dead, and most employers will understand the situation you're in–my employer did. I was able to take the time off for that. And I was going to cut my chances of finding a successful trial by limiting it to a small geographic area, say, within 50 or 100 miles. My goal was to find a clinical trial and qualify for it,

and then worry about the logistics of how to get there. One thing about most clinical trials is you're reimbursed for your mileage, [your] train ticket [if you have to buy one], your parking, [and] your tolls. [It’s] covered in clinical trials where it may require airfare to get there. Trials that are done at the National Cancer Institute or the National Institutes of Health handle all the logistics of travel. They pay for your airfare; if you have to stay over, that's all covered. There's no financial issues involved with that. But, with some of the other trials–say, you're going to a medical center. You're on the East Coast, and you find the trial on the West Coast. There are some organizations, one of which is called Corporate Air Angels, and there's several like that organization. They're non profits. They get donations–private donations–but also they have aircraft that are provided on a space available situation, so they'll provide a seat for both a patient and one caregiver. This organization will handle logistics, finding a corporate aircraft that's going from a corporate air park near you. If you can't get there on your own, they'll take care of having a Uber, Lyft or a taxi get you there. They fly it to [your] destination. They handle the ground transportation to where you're going to be treated. If they can't get you back on the same day on a returning aircraft, they'll make arrangements to put you up at a hotel and so on. There's absolutely no financial risk to the patient in doing this. It's just the time that's involved.

These are [the] kinds of things that I try to do. It's part of my advocacy to make people aware that there's a lot of things out there in helping you to get a better outcome. With that, I think if anybody wants to ask questions, I'm ready to go.

Brad Power 23:58
Thank you, Steven. Just a couple of definitional things, Steven–for those who don't know, can you describe the Whipple surgery that was step one in the standard treatment for your diagnosis?

Steven Merlin 24:21
The Whipple procedure is one of the most difficult surgical procedures to do. If you're of stage three or lower, they're able to do [the Whipple procedure]. Usually, when you have a tumor on the head of your pancreas, they can do the Whipple procedure. The procedure was developed in 1935 and has a pretty good outcome. [During the procedure], you're opened up underneath your rib cage, or they can do a midline vertical line down here. What they do is they resect the head of your pancreas. They take out the part of the bile duct and your gallbladder. They remove the duodenum, about two and a half inches of your jejunum, and the bottom portion of your stomach, although there is a pylorus preserving procedure where they keep the pylorus valve. Then they would attach the bottom of the stomach to a part of the jejunum, along with sewing on the remnant part of your pancreas, which would be the body and tail. Then, you're basically good as new. Some people may need to take digestive enzymes after having the Whipple [procedure] because in the head of the pancreas are the cells which produce and secrete your digestive enzymes, [like] amylase, lipase and protease. Sometimes you can have atrophy in the remaining part of your pancreas, and that could lead to becoming diabetic. That could happen about 30 to 40% [of the time].

I came through the Whipple okay; I didn't have any kind of issues afterwards, such as leakages where I had to have drains. I do take digestive enzymes, and the dose varies depending on the type of meal that I have. Generally, when you have meals that have a lot of dietary fat, that could be a problem for some and especially when you don't have a gallbladder where you have a reserve of bile salts to emulsify the fats in your diet. So if I stick to a low fat, non fat diet, I'm okay. I can get by with little to no Creon. But if I want to eat, as I did before I ever was diagnosed, which I do, [and] if I want to have a nice filet mignon or something like that, I'll take the Creon.

Basically, I'm back to normal. I'm a twelve and a half year survivor of stage four. I was told after the two years of the heavy chemo that [there] was no evidence of disease and I never did ask. But about 10 years out, I was at a meeting when a surgeon at Columbia University was doing a presentation and knew that I was in the audience, and was mentioning that there are now accounts of people that have done both FOLFIRINOX treatment and have [been cured], and mentioned me by name. I was totally shocked. I had no idea that was coming. And I've heard now from a number of other pancreatic cancer specialists around the country, they consider me cured, and they credit the amount of FOLFIRINOX, particularly the oxaliplatin component of it, that I had for doing that.

I do continue with the scanning. I also have what's called CT DNA, which is the cellular tumor DNA that's made specific[ally] for each patient. It's very sensitive. That test has been done since 2014. I have it done every 90 days, and it shows zero. They do the carbohydrate antigen 19-9 (CA19-9) test on me, although my tumor type never secreted that. But [say] I get another primary pancreatic cancer? It could secrete CA19-9, so that's the reason why we do that. And then, as I mentioned, I go twice a year for the [CT–actually, MRI scans–I have now]. Now, after seven years, I switched from CT to MRI because of the potential accumulated effects of the radiation and getting secondary cancers, so we do MRI.

Brad Power 28:59
Just one more quick thing following up on that. When you were initially diagnosed, what [was or is] the success rate of the Whipple procedure and the standard of care? What were you looking at?

Steven Merlin 29:14
The statistics now are that about 80% [of people] that have a Whipple [procedure] and finish their treatment within 24 months [experience] a recurrence. It's only about 20% [of people] that don't get a recurrence. And it's like you don't know who's going to get it. Knowing that, when I went into this, I was thinking,“Gee, we're doing the same thing over and over, and we're not getting really any better at it here… maybe we try something different.” That was the impetus for me to say, “Wait, we're only doing twelve cycles and we're seeing [an] 80% recidivism rate. Maybe I'll try going more than twelve to see what happens.”

Being that I came from a background in clinical research, I was always inquisitive. A lot of people that I mentor know my background, and they would ask me, “Well, why did they pick

twelve cycles of FOLFIRINOX?” I started doing searches on the internet, and I couldn't find that answer. I was really curious, and I had the opportunity to talk with one of the pioneers that did the early clinical trials on FOLFIRINOX. And I asked that question, and he was wondering how I knew to ask that. I told him about my background and [the] people [who] were asking me. And he said, “Well, most likely it was a working group of oncologists through the American Society of Clinical Oncology that were pancreatic specialists and asked the question, ‘How many cycles of FOLFIRINOX do you think your average patient could tolerate in order to achieve no evidence of the disease?’ And, by consensus, [the] number twelve was pulled out of the air.” Knowing that, the goal is to get to no evidence of disease. Well, what is “no evidence” of disease? Specifically, that is, by using conventional imaging of either CT, an MRI or a PET scan, they can't see disease. You have to realize that those instrumentations have a level of sensitivity. There's a threshold that, if it's not exceeded, won't see anything that's too small. You can have what's called micrometastatic disease or minimal residual disease, that of which remains after you go through treatment. It could be quiescent, and as long as your immune system is strong and robust, [your immune system] could withstand any kind of challenges. But if it weakens, all bets are off. Now anything that was quiescent could come back, and you now have the problem with having to deal with recurrence.

That was my impetus of “let's go further beyond the twelve [cycles].” Nobody knew how many [cycles] would result in cure or not. The reason why we stopped at 46 [cycles] was because at that point, the clinical trial that I found came [back to me]. The response to my tumor was slowing, and it may have been because there was nothing left. All they could see [was that] I had six large tumors scattered throughout my pancreas, and then there were others that were classified as TNTC (too numerous to count), so they were all over. A lot of them were below one centimeter. My largest tumors–one was 4.2 by 3.8 centimeters. There was another one almost as large, sitting right on top of the hepatobiliary tree, so that made it inoperable. I was looking at other types of techniques such as radio frequency ablation. Now they have what's called “NanoKnife", or irreversible electroporation that wasn't available twelve years ago when I was around, or [else] I would have considered that. At that point, my oncologist said, “Okay, it looks like you're on the waning side of the chemo. Now's the time to switch and go into the clinical trial.”

The purpose of the clinical trial that I was in, using a PARP inhibitor, is for maintenance monotherapy. For people that have BRCA mutations, there's a slightly increased risk of developing a new primary cancer. For males with BRCA mutations, it's pancreatic, male breast cancer and prostate cancer. For women, it's ovarian, pancreatic, and breast cancer. Knowing that, I went on this PARP inhibitor; the trial was for two years. I was the first US patient on it. I was on it the longest. I had a complete response to it, and I'm still on it 10 years and three months later. They don't know whether to stay on or not [at this point], because there's nobody ahead of me to know what happens if you go off it. It was left up to me, and I figured being that I have an increased risk, and there were no issues with suppressing my bone marrow–where they found in women that were treated with carboplatin, which is a type of platin agent (I had oxaliplatin, which is not quite as harsh.), some of those women went on to develop myelodysplasia syndrome, which led to Acute Myelogenous Leukemia, which could be fatal. Of

course, that was a concern of mine. I did have a bone marrow aspirate done and found that there was no effect. And usually that happens earlier in your course of treatment, within PARP inhibitors. I did a lot of research into that. Everything was fine, so I've stayed on it. Hopefully, it's giving you protection against developing prostate cancer and male breast cancer, as well as a recurrence of pancreatic cancer.

David Plunkett 35:34
When you did the genetic testing, how did you arrange for that? And is there anything better available now?

Steven Merlin 35:46
Good question. When I was telling my oncologist the history of the family with breast cancer, [he] asked [me] if I had ever had genetic testing. [As I] said, “No,” he goes, “Okay, I'm going to send you over to the geneticist at Weill Cornell.” I met both with the geneticist and actually I first met with a genetics counselor. [The geneticist] did a cheek swabbing, and they extracted the DNA from that, and they did the family pedigree. I had a pretty good idea of all the people in the family, both the maternal and paternal side, and going way, way back to great grandparents, distant cousins, and so on. We could see what the history of cancer was [like] there. But for people that have a history of any kind of cancers in the family, and especially if it's same type of cancer, you can either go through your primary care physician and say, “Hey, there's a history of cancer in here, especially in close blood relatives,” and they can refer to you to a geneticist. Or, you could call up the genetics department in a large medical center. Generally, if it's a National Cancer Institute, Center of Excellence, or a comprehensive cancer center, they will likely have a genetics department. You call them up and tell them that there's a history of cancer in the family, that you're concerned, and that you're near an age where a member of the family had developed the cancer that you'd like to go in for testing. And it's covered by insurance, so there's no issues there. They could do it on those three things, and it generally takes about two weeks–

[STEVEN MERLIN MOMENTARILY DISCONNECTS DUE TO TECHNICAL ISSUES]

Roger Royse 39:03
Did [Steve Merlin] say he had his tumor tissue checked for mutations?

Rick Davis 39:22
He definitely did. He's had loads and loads of NGS and similar, but ask him about it, and he'll tell you. But he's had a lot of–as far as I recall–he's done a lot of analysis on his tumor.

Roger Royse 39:43
Did he say which PARP inhibitor he's taking?

Brad Power 39:47
No, he did not say.

Rick Davis 39:50
At this point, although there are trials out there–in fact, I got a buddy in Sydney, Australia, a doc who was just going into one for a new part. But the existing parts don't seem to make much of a difference. They differ in terms of their potency, so the size of the dose changes, but in terms of the actual effect, we don't seem to think that whether you're taking rucaparib or [*indistinct*] it makes a difference.

[STEVEN MERLIN RECONNECTS]

Steven Merlin 40:34
The PARP inhibitor I'm taking is called Rubraca. It was known as rucaparib. They never applied for FDA approval on that because AstraZeneca had the market share with Lynparza (a PARP inhibitor), also known as olaparib. Rucaparib is used in women with ovarian cancer, and it also got approval in prostate cancer, and it has a very good safety profile. I have really no side effects from it, other than anemia. I did one dose reduction, and that's how we manage it. About every six to seven months, I'll go on a short pause, and all my hematology parameters bounce back up into the normal range.

Steven Merlin 41:50
The other question I heard was, did I have the somatic testing done? That was done on a tissue sample that had been archived after the surgery, and nothing came up other than [that] I have just BRCA2 mutations or no other mutations that were the driver of my cancer.

Bill Paseman 42:12
And this is all DNA seq, not RNA Seq, correct?

Steven Merlin
Right, correct.

Bill Paseman
What kind of scans are you taking in annually, or how ever often you do?

Steven Merlin 42:22
Now, every six months I do MRIs. For the first seven years, I was doing CT, with and without contrast, and then a number of [oncology scans]. You were saying, seven years–that's a lot of accumulative radiation. I was concerned about getting secondary cancers, especially lung cancer, from it, so they recommended switching over to MRI. The problem with MRI is that you can't image lung tissue. They could do low dose. I'm getting CtDNA done. I'm getting CA 19-9 done. We're just doing the MRIs. If anything is detected, then we could do a lung screening. But the risk right now, at 12-and-a-half years out, is pretty low.

Roger Royse 43:22
Is that an MRI with gadolinium?

Steven Merlin 43:25
Yes. Of course, there's people who raise the issues about accumulative gadolinium and brain and so on, and I checked with the FDA: they're aware of these concerns, and they're monitoring this. But so far, there's been no pathologies associated with the gadolinium.

Brad Power 43:53
Roger, did you have any other questions?

Roger Royse 43:55
I have so many questions, but let me just ask a couple of them, just quickly. The test you get every 90 days is cfDNA. Is that Signatera?

Steven Merlin 44:06
No, I'm getting ctDNA. It's done by another company–it was in conjunction with the clinical trial that I was on. I don't know the name of the company, but I I met one of the principals of the company, and they're doing a study at the University of Pennsylvania. [They are] looking at patients and their ctDNA to see if there's any changes early on [and] trying to detect recurrence.

Roger Royse 44:36
Okay, and it surprises me; you're getting scans every six months, 10 years out. I thought the standard of care was [that] they stopped scanning after five or six years, and they go to one year scans three years out. How do you get so many scans?

Steven Merlin 44:55
I asked for it, and I said “I know the high rate, I went through the hell of a Whipple and all of that chemo.” And it's like, I'm not throwing caution to the wind. And if clients can learn anything from my participation in doing more of the scans and stuff, I'm happy to be able to do that. For me, it's not inconvenient for me to do the scans. I do them at where I had worked at Weill Cornell, but all of my care now is through University of Pennsylvania. Every six months I go into New York and have the scans done.

Roger Royse 45:34
That's one of the surprising things about this. I had to negotiate my scans as well, like everything else, because they wanted to do fewer than I wanted them to do at first, but you would think, “I don’t know very many people would think to even ask.” The doctor says, “Gee, the insurance company says you should have this.” How many people are going to say, “Well, I think I'd rather do something else!” Last question I have is, you were stage four–you said metastatic. Was [that referring] to your liver?

Steven Merlin 46:11
Yes, just in the liver. I was actually stage four when I had the Whipple and they didn't know it. Back in 2012, the resolution of a CT [scan] was between four and five millimeters; today it's down to 1.25. I may have just been below the threshold of detection at that point. Two weeks later, when I had the original diagnostic scan, is when the radiologist started to see hints that I

likely had metastatic disease to the liver. The tumor board, knowing that, put me on a very mild chemo[therapy] called Gemzar or gemcitabine. And back in 2012, Abraxane was still in clinical trials, so it wasn't the combo drug yet, and basically, Gemzar was a little bit better than taking water. With me, it turned out that I was one of the 10% in the population that [isn’t able to] make the enzyme to metabolize Gemzar. [My body] was never breaking it down, and I had no toxic effects to my malignant cells. I remember when they would ask me, “How are you feeling?” and I’d go, “Yeah, it feels like I'm doing water,” and now I can laugh at that, because basically I was getting the same effect as drinking water. It had absolutely no effect. The next time we had that CT scan, the tumors were significant in size, and they were all over my liver. It was lucky that I responded to the FOLFIRINOX.

The other reason why I still do two scans a year is that, in 2018, they discovered that I had an interductal papillary mucinous neoplasm in a branch duct in the tail of my pancreas. Generally, when they're in the branch duct, they're benign, and they have found another one which is in the body of my pancreas, but it's also in a branch duct. For peace of mind, I go for surveillance every six months. We do an MRI, and I have the consult at Memorial Sloan Kettering in New York, but all my scans I've always done at Weill Cornell, so I've kept that consistency of basically the same instrumentation and the same radiologists that are familiar with my scans. And I also make sure that I point out that, when they ask “what is the purpose of getting the scans?” that I tell them it's a long term follow-up, having had metastatic disease to my liver, and also that I have intraductal papillary mucinous neoplasms, so that they're focusing in on those two things when they're reading my scans.

Roger Royse 49:01
On the PARP inhibitors–I've heard that you people develop resistance pretty quickly to those. But you've been on this for 10 years, it sounds, so you must not think so, right?

Steven Merlin 49:16
Yeah, a lot of clinicians are surprised. I was at scientific meetings, and they were talking about how PARP inhibitors in generally two years and three maximum, most people would get resistance to it. And I walked up to the microphone and introduced myself, and I think, at that time, I had been on it for eight years, and they were all really surprised. There are people that are statistical outliers out there: I'm one of them. I was on full dose of the rucaparib for six years, until, one day I noticed I was doing some really strenuous work and felt extremely fatigued. I got very lightheaded. I started to get syncope, I thought I was going to hit the ground, and I mentioned it to my oncologist–we did the hematology check. My hemoglobin went down to 7.1. When you drop below seven, they give you a transfusion. That's how close I was.

Walking up a flight of stairs was extremely debilitating. If I suddenly got up out of bed, I would get very lightheaded and almost pass out. That's when I went on a six week break from it. Everything bounced back up. Another thing that PARP inhibitors do is they will give you a decrease in your estimated glomerular filtrate rate, so your EGFR will go down. You'll see your creatinine rise. And then first thing, a nephrologist will look and say, “Oh, you've got stage three ‘A’ kidney disease here,” and they wanted to classify me as stage three “A.” Well, it turned out,

when I took the short pause from the PARP inhibitor, my EGFR went back to normal. My creatinine dropped back down to normal; my hemoglobin, hematocrit and red blood cell levels all increased. It's just transient changes that I see when I'm on it long term. We decided to manage me long term. We did one dose reduction. Originally, I was at 1200 milligrams, I went to 1000, and now, instead of doing another dose reduction, I just do pauses every six to seven months, and I feel great again. I just came off of a recent pause. I've been back on the PARP inhibitor for a week now, so I'm still feeling great. But slowly over the next five to six months, I'll start to feel it a little bit, and I'll usually stop before it gets really bad, and take that short pause.

Bill Paseman 52:23
What are the genetic mutations you wind up having? Has anybody else–you have a unique signature–has anyone else tried this medication on it? And they mentioned that this drug stops working–maybe it has stopped working, maybe, basically, you've had a complete response. That's my one question.

Steven Merlin 52:38
Right, so there are a couple of PARP inhibitors: Zejula, also known as niraparib, Lynparza, which is olaparib, and rucaparib. There's a few others. They're working on the next generation and also the clinical research is focusing on trying to figure out how to prevent the resistance to PARP inhibitors.

Alane Watkins 53:06
Have you been following the clinical trials at NIH for histotripsy on the pancreas, and do you know of any initial successes on that?

Steven Merlin 53:17
Yes, I know in my area, NYU and also Weill Cornell, is doing histotripsy. Now, a number of places are coming online around the country. There really isn't any long term data yet on pancreatic cancer, but it's pretty much a non-invasive procedure, and looks like a good alternative than going in there and having to do resection of your liver. How it compares to NanoKnife, I really don't know yet, so I'm waiting to hear more. Hopefully information will be forthcoming from these clinical trials to see how they compare head to head. It's something I would definitely consider if I ever had a recurrence in the liver.

Rick Davis 54:10
I was just wondering if you've ever repeated your NGS (next generation DNA sequencing), because, in cancers like prostate and others, the somatic mutations can change over time. I don't want anybody to walk out of here thinking, “Oh, I should only get it one time.” You have to keep monitoring.

Steven Merlin 54:31
Exactly. This was a question I had. I asked about how frequently we should do this. Generally, I say that about every four to five years, there's many more new gene mutations discovered that

it would make sense [to keep monitoring]. I don't have any more tumors that they could go in there and take a look. We just have to do it by blood tests to see what they can detect in that way. I've had it done three times now over the past 12 years.

Ellen Miller 55:09
Do you have PDAC, or do you have the endocrine, or do you have a different kind of pancreatic cancer?

Steven Merlin 55:26
Primarily, it's pancreatic acinar cell carcinoma, but it was a mixed tumor type, so I also had PDAC, but the vast majority was the acinar cell type, which is slightly better than PDAC.

Ellen Miller 55:41
Okay. And then the other thing I wanted to ask you about: What is your approach to lifestyle choices, especially with diet, exercise, that sort of thing. Xo you just eat whatever the heck you want, or do you do anything special?

Steven Merlin 55:56
Yeah, great question. I was always very health conscious since I was in high school. When I started eating non fat, low fat foods, I did a lot of exercising, and at the time of my diagnosis, I was doing extensive bike riding for endurance. I would do 50 mile segments as much as four times a week. I was in great shape–my diet, everything else–but you can't do anything about genetics. Have I changed anything? No, I still maintain that healthy lifestyle. I do eat a little bit more red meat than I used to, and sometimes I'll add some chocolate cake, things like that, whereas I would pretty much avoid sugar up until I got ill. I don't deprive myself if I want something sweet; I don't worry about a bowl of ice cream, things like that. But otherwise, I maintain a healthy diet and exercising. I try to do at least 150 minutes of exercise per week. It could be 30 minutes a day. Sometimes I'll do a three mile or four mile walk, which will take me an hour. Even in cold weather, if it's really inclement weather, I have a stationary bicycle: I'll do that. Or if I'm some place where there's a multiple story building, I’ll walk up the stairwell several times to do cardiovascular. I'm cheap, I don't go to the gym. I find all these free things that you can do, like free Stairmasters in a building,

Brad Power 57:35
What's your summary or any closing message you'd like to share with us?

Steven Merlin 57:44
You need to do your research, and don't be afraid to speak up. If you find things, present that to your care team. Have the information with you.

Always write down things. I’m a big advocate of using the patient portal, so that between my follow-up exams, I'll make notes and put it in there to make them aware of how I'm feeling, so that they can then address it when I go there, in case I forget it.

“How Advocacy Leads to Better Patient Outcomes and Experiences” (Steven Merlin) [#126]

Always stay on top of things. Stay attuned to your body. If something's not right, don't say, “I'm going to wait and see if it goes away.” If it hasn't cleared up within five days, go see your primary care specialist.